PR-07 Pertussis response protocol

Southern Inland Public Health Unit, Southern Inland Local Health District

Version	Effective	Owner	Approved	Next review
3.1	02/03/2026	Director, Southern Inland Public Health Unit	Director, Southern Inland Public Health Unit (Nathan Olson)	March 2028

1 Purpose and scope

1.1 This protocol sets how the Southern Inland Public Health Unit responds to pertussis notifications: case follow-up, where the outbreak response team works, school clusters and infant cases.

1.2 It covers residents of the unit's ten local government areas: Wagga Wagga, Griffith, Leeton, Narrandera, Murrumbidgee, Hay, Edward River, Junee, Cootamundra-Gundagai and Snowy Valleys. A notification for a person living elsewhere is referred to the public health unit for that person's area and is not counted under this protocol.

2 Case definition

2.1 A confirmed case has laboratory definitive evidence: *Bordetella pertussis* detected by nucleic acid testing (PCR) or isolated by culture.

2.2 A probable case has laboratory suggestive evidence (*B. pertussis* IgA antibody detected) and clinical evidence, or clinical evidence and an epidemiological link to a confirmed case.

2.3 From 1 July 2024 a case whose only laboratory evidence is serology is probable; before that date the state case definition counted it as confirmed. The case status on a notification is the status assigned when the notification was received.

2.4 Counts under this protocol include confirmed and probable cases.

3 Notifications and cases

3.1 Laboratories notify the state notifiable conditions system, and the unit's line-list extract holds one row per laboratory notification. A person tested by two laboratories, or tested again, is notified more than once.

3.2 Where a case's onset date is not recorded, epidemic curves take it as the specimen collection date less 10 days.

3.3 Notifications for the same person with specimens collected within 90 days of the first specimen of an episode belong to that episode and are one case. The case is confirmed if any of its notifications is confirmed. A specimen collected more than 90 days after the episode's first specimen begins a new case.

3.4 A case's area is the local government area of its residential postcode, through the unit's postcode lookup. Where a postcode is split between areas, the case is allocated to the area holding the larger share of the postcode's population.

3.5 Every count under this protocol dates a case by the collection date of its first specimen.

4 Deploying the outbreak response team

4.1 The outbreak response team works in one area each week. The area is set at the Monday huddle from the line-list extract run at 07:00 that morning.

4.2 The team goes to the area with the largest excess of cases with specimens collected in the 14 days before the extract day over its expected count.

4.3 An area's expected count is the mean, over the previous five years, of its cases with specimens collected on the same 14 calendar dates.

4.4 The team leader confirms travel, clinic sessions and school visits with the area's council, general practices and principals by the Tuesday of the deployment week.

5 School clusters

5.1 A school cluster is two or more cases among the students of one school with specimens collected in the 30 days before the extract day. A student is a case recorded at interview as a student of that school. The huddle slide lists the clusters in the area the team goes to, each with the school and its number of cases; they are the team's school visits that week (4.4).

5.2 For each cluster the unit sends the school the pertussis information letter, asks for the class rosters and reviews the vaccination records of the cases' classmates.

6 Infants

6.1 An infant case is a case aged under six months on the collection date of its first specimen.

6.2 An infant case was admitted to hospital when the respiratory admissions extract holds an admission for the infant, matched on medical record number, that began within 14 days before or after that collection date.

6.3 An infant case was born to a vaccinated mother when the antenatal vaccination register records a pertussis-containing vaccine (dTpa) given to the mother during that pregnancy at least 14 days before the birth. Infants are linked to their mothers through the birth record.

7 Reporting

7.1 The quarterly report gives, for each area, the period's confirmed cases, probable cases, infant cases, infant cases admitted to hospital and infant cases born to a vaccinated mother.

Document history

Version	Date	Change
3.1	02/03/2026	School clusters (section 5) and the huddle slide; team leader confirmations (4.4)
3.0	18/11/2024	Expected count over the previous five years (4.3); split postcodes (3.4)
2.2	01/07/2024	State case definition revision: serology-only cases probable (2.3)
2.0	14/02/2022	Infant section (6) and quarterly reporting (7)

Distribution: outbreak response team; surveillance; immunisation; public health nursing. Printed copies are uncontrolled; the current version is on the unit's shared drive.